

# Excellence Health System

## Appeal Request — Reconsideration of Adverse Determination

**Patient Name:** Carlos Mendez

**DOB:** 1964-05-22

**MRN:** 63928417

**Original Denial Date:** 2026-08-01

**Member ID:** MBR-00733

**Appeal Filed Date:** 2026-08-10

**Appeal Level:** Level 1 — Internal Appeal + Grievance

### REASON FOR APPEAL

This appeal requests reconsideration of the determination to classify the patient under Observation Status rather than Inpatient Admission for evaluation of acute chest pain with concern for unstable angina.

### CLINICAL RATIONALE FOR RECONSIDERATION

While initial cardiac biomarkers were negative, the clinical presentation warranted inpatient-level monitoring:

- Patient is a 62-year-old male with prior MI (2024), HTN, HLD, and active smoking
- Presented with substernal chest pain radiating to left arm, with diaphoresis
- Initial EKG showed ST depression in leads V4-V6 (new compared to prior)
- Troponin I: 0.03 ng/mL at presentation (borderline — repeat was 0.08 at 4 hrs)
- Patient required IV Nitroglycerin drip for refractory pain
- Cardiology recommended emergent stress test and possible cardiac catheterization

### POLICY REFERENCE

Per policy UM-OBS-IP-001 (Observation Status vs Inpatient Admission):

- Criterion: Need for IV medications > 24 hours — MET (IV Nitro drip x 36 hrs)
- Criterion: Serial monitoring with expected stay > 48 hours — MET (72-hr stay)
- Criterion: Invasive procedure anticipated — MET (cardiac catheterization performed)
- Criterion: Clinical instability — MET (dynamic EKG changes, rising troponin)

### REQUESTED SERVICE

Reclassification from Observation Status to Inpatient Admission retroactively, effective from the date of ED presentation (2026-08-01).

### KEY EVIDENCE CITED

- ED Provider Note dated 2026-08-01
- Serial EKG Reports (2026-08-01, 2026-08-02)
- Serial Troponin Results (0.03 -> 0.08 -> 0.15 ng/mL)
- Cardiac Catheterization Report dated 2026-08-03
- Cardiology Consultation Note dated 2026-08-01

### FINANCIAL INFORMATION

Estimated cost of requested service: \$32,400.00

Observation status cost-share to patient: \$8,600.00

Inpatient admission would reduce patient responsibility to: \$1,200.00

Financial exposure / disputed amount: \$7,400.00

Diagnosis Category: Cardiology — Acute Coronary Syndrome

### **APPELLANT INFORMATION**

Appellant Type: Patient / Member Representative

Patient Representative: Maria Gonzalez (daughter, authorized representative)

Contact: maria.gonzalez@email.com | Phone: (555) 456-7890

### **GRIEVANCE STATEMENT**

The patient's family files this grievance citing financial hardship caused by the observation status classification. The patient underwent cardiac catheterization and a 72-hour hospital stay. Classification as observation status resulted in significantly higher out-of-pocket costs that would not have applied under inpatient.

### **CONCLUSION**

We respectfully request retroactive reclassification to inpatient status. The clinical course — including rising troponin, dynamic EKG changes, IV drip requirement, and emergent catheterization — clearly met inpatient admission criteria under UM-OBS-IP-001 from the time of initial presentation.